

PATIENT MEDICAL PROFILE: PRIYA IYER

General Information:

- Full Name: Priya Iyer
- Age: 34 years old
- Gender: Female
- Nationality: Indian
- Primary Language: Tamil, English, Hindi
- Contact Number: +91 98989 89898
- Emergency Contact: Anand Iyer (Spouse) - +91 98989 89897

Clinical Presentation:

- Chief Complaint: Severe abdominal pain starting around the umbilicus and shifting to the Right Lower Quadrant (RLQ). Accompanied by nausea, loss of appetite (anorexia), and two episodes of vomiting.
- Duration: Pain started 12 hours ago, shifting to RLQ and worsening over the last 4 hours.
- Severity: 7/10 pain, aggravated by walking or coughing (positive rebound tenderness suspected on examination).

Medical History:

- Chronic Conditions: Mild Irritable Bowel Syndrome (IBS-C).
- Gynecological History: Last Menstrual Period (LMP) was exactly 3 weeks ago (regular cycles). No history of pelvic inflammatory disease.
- Surgical History: None.

Current Medications:

- Dicyclomine 10 mg orally as needed for abdominal cramps/spasms (took 1 dose 6 hours ago with no relief).

Allergies:

- No known drug allergies (NKDA).

Triage Vital Signs:

- Blood Pressure: 110/72 mmHg (Normal)
- Heart Rate: 88 bpm (Normal)
- Respiratory Rate: 16 breaths per minute
- Temperature: 100.2°F (37.9°C) (Low-grade fever)
- Oxygen Saturation (SpO2): 99% on room air

Triage Assessment & Emergency Department Protocol:

- Suspected Diagnosis: Suspected Acute Appendicitis / Acute Abdomen.
- Recommended ESI Triage Level: ESI Level 3 (Patient is hemodynamically stable, but requires multiple resources: labs, IV fluids, imaging, and surgical consult).
- Required Immediate Interventions:
 1. Maintain NPO status (Nothing by mouth) in anticipation of potential emergency surgical intervention.
 2. Establish IV access and start IV maintenance fluids (e.g., D5 1/2 NS or NS).

3. Draw blood for CBC (assess for leukocytosis), CMP, CRP, and a mandatory Beta-hCG (pregnancy test) to rule out ectopic pregnancy.
 4. Obtain a urine sample to rule out urinary tract infection or nephrolithiasis (kidney stones).
 5. Administer IV analgesics (e.g., Paracetamol 1g IV) as ordered once assessment is confirmed.
 6. Schedule abdominal ultrasound or CT scan.
- Handoff Instructions: Keep patient comfortable in bed. Do not apply heat to the abdomen (risk of appendix rupture). Re-evaluate pain score and temperature.